

TM-002: TREATMENT ARC + SESSION 1 FULL SCRIPT

TM-002 (T.M., 22) | TRAUMA-001 | CSP Pro Bono | THERAPIST ONLY

TREATMENT ARC: 6-8 SESSION FLOW

All 6 presenting themes (trauma, self-esteem, confidence, trust, anxiety, letting go) share a single root: childhood trauma. The arc works from root outward to symptoms. EP result from Session 1 determines delivery lane for the entire program.

SESSION	FOCUS	THERAPEUTIC GOAL	KEY TECHNIQUE	BUILD ON
S1	Assessment + First Induction	Screen, determine EP, build rapport, establish safe space	Intake (L.O.V.E.), 36-Q suggestibility test, Theory of Mind, first induction, safe space visualization	Foundation
S2	The Root: Inner Child Encounter	Access the core wound. Adult meets child in the Room. Identify the belief that got hardwired.	Room Technique Phase 1-3 (build room, meet child, begin re-parenting). NOT regression. NEW safe room.	Safe space from S1
S3	Re-Parenting + Core Belief Reframe	Replace "I'm not safe / not enough" with "I am safe, I am enough, I always was"	Room Technique Phase 4-5 (address wounds, give confidence). Core belief installation. Memory reconsolidation.	Child relationship from S2
S4	Integration + Anxiety Dial-Down	Merge inner child with adult. Recalibrate threat detection system.	Room Technique Phase 6 (integration). Anxiety dial/dimmer. Nervous system recalibration. Vagal toning.	Re-parenting from S3
S5	Trust + Assertiveness + Boundaries	Open the circle of trust. Install the assertive voice. Boundary anchoring.	Circle of Trust visualization. Assertiveness chair. Boundary wall with gate (client controls who enters).	Integrated self from S4
S6	Self-Hypnosis + Independence	Client runs the protocol alone. Reinforce all installations. Graduate.	Self-hypnosis teach (safe space + dial + affirmation sequence). Relapse prevention. Future-pace.	All prior work
S7-S8	Contingency / Reinforcement	Only if needed. Address anything that surfaced. Reinforce weaker installations.	Based on progress data.	As needed

Session-to-session decision points:
After S1: EP determines Physical (direct reframe) vs Emotional (gentle discovery) lane for entire arc.
After S2: If child encounter triggers strong catharsis, slow down. Add a stabilization session before S3.
After S3: If core belief hasn't shifted, repeat. Don't advance to integration until the reframe has landed.
After S4: If anxiety dial-down is incomplete, add reinforcement before trust work in S5.
After S5: If client is self-sufficient, graduate at S6. If not, S7-S8 available.

KAPPAS RULE: Visit the past briefly to identify the root belief. Recode it in present tense. Move forward. Don't loop. The goal is BEHAVIOR CHANGE in the present, not endless childhood processing.
MELTON RULE: Check expectations at every session. "What are you hoping will happen today?"
NASIR RULE: The limbic system doesn't respond to insight. It responds to experience. Trance provides the experience that talk therapy cannot.

SESSION 1: FULL SCRIPT

[Client: T.M., 22, childhood trauma, 6 presenting themes. CSP pro bono. Prior psychotherapy. Requested male therapist. EP: TBD. This session is ASSESSMENT ONLY. No deep trauma work. No regression. Build the foundation.]

PART 1: RAPPORT + INTAKE (20-25 MIN)

Opening (warm, direct, set the tone)

T.M., welcome. Thank you for reaching out through the community service program. Before we get into anything clinical, I just want to check in. How are you doing today?

[Let him talk. Listen for energy level, comfort, nervousness. Do NOT rush. First 2-3 minutes are pure rapport.]

Good. So here is how tonight works. This is our intake session. Think of it as the blueprint session. We are going to talk about what brought you here, I am going to explain how hypnotherapy works and how it is different from the psychotherapy you have already done, and if you are comfortable, we will do a first relaxation experience at the end. No pressure to go deep tonight. Tonight is about understanding what we are building together.

Sound good?

L.O.V.E. Intake (Listen, Observe, Validate, Explore)

Tell me what brought you here. In your own words.

[LISTEN. Do not interrupt. Let him tell the full story. Note the exact words he uses for each theme. His language reveals his subconscious framing. Watch for: body language shifts, vocal tone changes, eye contact patterns, emotional loading on specific words.]

Thank you for sharing that. That takes courage, especially with someone new.

[VALIDATE. Do not minimize. Do not say "I understand" unless you genuinely do. Better: "That makes sense" or "Thank you for being honest about that."]

I want to make sure I heard you correctly. You have been working on this in psychotherapy, and it has helped a lot. But there are some pieces that talking about it has not been able to fully shift. The self-esteem, the trust, the hypervigilance, the feeling of not being comfortable in your own skin. Those are still running in the background. Is that right?

[REFLECT back. Let him correct you. His corrections tell you what matters most.]

And you mentioned wanting to become more open and less hypervigilant. Can you give me a specific example? A recent situation where you noticed the hypervigilance or the anxiety running the show?

[EXPLORE. Get a concrete behavioral example. This grounds the work in real life (Kappas: focus on behavior, not abstract healing). Also reveals which theme is most active RIGHT NOW.]

Checking Expectations (Melton)

Before we go further, I want to ask you something important. What are you expecting from hypnotherapy? What do you think will be different about this compared to the psychotherapy you have done?

[IMPORTANT. His answer tells you what matters most and what to prioritize. Understanding and change are not mutually exclusive. Listen for what he wants to be different.]

That is a great answer. Here is how I think about it. Psychotherapy gave you conscious insight. You understand what happened and why it affected you. That is real progress. But the reactions you are describing, the hypervigilance, the trust issues, the anxiety running the show, those do not live in the conscious mind. They live deeper. In the subconscious. In the part of your brain that was programmed when you were a kid.

Think of it this way. The therapy you have done gave you understanding and coping tools. Those are real. What we are going to do is add an experiential layer. In a deeply relaxed state, your mind becomes more open to experiencing things differently, not just understanding them. We are building on what you have already accomplished.

[Validate his prior therapy work. Position hypnotherapy as complementary, not corrective. Do NOT claim psychotherapy "could not reach" something.]

What do you already understand about these reactions, and what would you like to be different in how you feel or respond?

And what are you hoping hypnotherapy will add to the work you have already done?

[Listen for what matters most to him. Understanding and change are not mutually exclusive. His answer tells you what to prioritize, not whether he is "ready."]

PART 2: CRITICAL SCREENING (5-10 MIN)

[DO NOT SKIP ANY OF THESE. Childhood trauma requires thorough screening before any trance work. Ask directly. Do not soften.]

Before we go any further, I need to ask you some screening questions. These are standard for every client, but especially important given what you have shared. I need honest answers so I can make sure we work safely.

Question 1: Active Professional Care

Are you currently seeing a therapist, psychologist, or psychiatrist? Are you in active treatment right now?

[If YES: get name, type of care, how often. Coordinate, do not conflict. If psychiatrist: check medications. If NO: note that psychotherapy was past, not current.]

Question 2: Medications

Are you currently taking any medications? Anything for anxiety, depression, sleep, or anything else?

[If anti-psychotic: STOP. Refer to licensed MHP. Do not proceed with hypnosis. If anti-anxiety or anti-depressant: proceed with awareness. Note medication name and dosage.]

Question 3: Safety

I need to ask this directly. Have you ever had thoughts of harming yourself or ending your life?

[If ACTIVE suicidal ideation: STOP the session. Provide 988 Suicide and Crisis Lifeline (call or text 988). Do not proceed with hypnosis. Refer to licensed MHP. If PAST ideation (resolved): note it, proceed with caution.]

Question 4: Trauma Scope

You mentioned experiences from when you were young. You do not need to share details today. But can you help me understand the scope? Was this a single event, or something that happened over time? And were you ever physically harmed?

[Scope the trauma WITHOUT forcing disclosure. Session 1 is assessment, not processing. You need to know: single vs repeated, physical vs emotional vs both, who was involved (family, outside). This determines the intensity of future sessions.]

Question 5: Triggers

Is there anything I should be careful about? Words, topics, types of imagery, or situations that tend to trigger strong reactions for you?

[ESSENTIAL for trance work. If he says "closed spaces" do not use confined visualizations. If "authority figures" adjust your tone. If "physical touch imagery" avoid body-scan language. Write these down.]

Question 6: Panic

Do you experience panic attacks? How frequently?

[If yes: adjust induction. Use open-space imagery, not enclosed. Include grounding anchors. Give him a signal to use if he needs to come out of trance ("raise your right index finger if at any point you need to pause").]

Thank you. Those are the hard questions. You handled that well. Everything you shared stays between us, with one exception: if you are in immediate danger, I have a legal obligation to act. Otherwise, this is confidential.

[CSP note: add supervision disclosure.] "One thing to be aware of: because this is through the community service program, I may discuss our session with a clinical supervisor for educational purposes. Your identity is always protected. I would only use your initials."

PART 3: SUGGESTIBILITY TEST (10-15 MIN)

Now I want to understand how your mind processes information. This is not a test you pass or fail. It is a diagnostic tool that tells me how to communicate with your subconscious most effectively. There are two main styles: Physical and Emotional. Everyone is a mix. I need to know your mix so I can tailor everything we do to how your brain is wired.

[Administer the 36-question suggestibility test. Score it. Determine Physical/Emotional percentage. This result determines the delivery lane for the ENTIRE treatment arc.]

Alright, let me score this.

[Score and announce result. Explain what it means in simple terms.]

IF PHYSICAL (direct, literal):

"You scored [X]% Physical. That means your subconscious responds best to direct, specific language. When I say 'your hand IS rising,' your hand rises. When I say 'you ARE relaxing,' you relax. Literal, concrete, no ambiguity. That is how we are going to work."

IF EMOTIONAL (permissive, inferential):

"You scored [X]% Emotional. That means your subconscious works through inference and feeling. Instead of me telling you what to experience, I suggest possibilities and your mind fills in what is right for you. 'You may notice a sense of warmth...' and your subconscious takes it from there. That is how we are going to work."

PART 4: THEORY OF MIND + EDUCATION (10-15 MIN)

Conscious vs Subconscious

Let me explain how this process works and how it builds on what you have already done in therapy.

You have two minds. The conscious mind is the one you are using right now. It analyzes, reasons, makes decisions. It is about 12 percent of your mind's processing power. The other 88 percent is the subconscious. It runs automatically. Your heartbeat, your breathing, your emotional reactions, your habits, your beliefs about yourself. All subconscious.

When you were a kid, experiences got programmed directly into that 88 percent. No filter. No analysis. Just raw encoding. The beliefs that got hardwired, things like "I am not safe," "I cannot trust people," "I am not enough," those are not thoughts you chose. They are programs that were installed by experience.

The therapy you have done already made real progress. You said so yourself. That work gave you understanding, coping tools, and the ability to talk about what happened. Those are genuine gains. What you are describing now, the reactions that are still there despite that understanding, those often involve patterns that were encoded very early, before you had words for them. They can be harder to reach through conversation alone.

What we do in hypnotherapy is work with a deeply relaxed state where your mind is more open to experiencing things differently. Not just understanding them, but feeling them shift. Think of it as adding an experiential layer to the insight you already have. We are not replacing what therapy gave you. We are building on it.

Fight-Flight-Freeze (Nervous System)

Here is another way to think about it. Your nervous system has a threat detection system. It was designed to protect you. When you were a kid and things were not safe, that system went on high alert. It learned to be hypervigilant because hypervigilance kept you safe.

The problem is, it never turned off. You are 22 now. You are safe. But the alarm system is still running at the same level it was at when you were five. That is not a character flaw. That is a survival program that did its job and does not know it can stand down.

Part of what we are going to do is teach that system that it is safe to turn down the volume. Not turn it off. You still need it. But right now it is at a ten. We want to bring it to a two or a three. Appropriate vigilance, not hypervigilance.

AI Disclosure

One more thing before we begin the relaxation. I want to be transparent about my preparation process. I use AI-assisted tools to research best practices, prepare session materials, and track clinical notes. All the therapeutic decisions, what techniques to use, how to adapt them for you, those are mine. The AI is a preparation tool, not a therapist. I mention this because transparency matters to me.

PART 5: FIRST INDUCTION + SAFE SPACE (15-20 MIN)

TRAUMA-SPECIFIC RULES:

- NO age regression. No "go back to a time when..."
- NO forced imagery. If he resists a visualization, follow his lead.
- Include permission: "with your consent," "only as far as comfortable"
- Build a safe space that HE designs, not you
- Give him a signal: "If at any point you want to pause, raise your right index finger"
- If abreaction: deepen trance, guide to safe space, do NOT count out abruptly
- PHS wording: add "safely and comfortably"

Alright. Are you ready to experience what this feels like?

Here is what is going to happen. I am going to guide you into a state of deep relaxation. You will be aware the entire time. You can hear me, you can think, you can open your eyes if you want to. You are in control. All I am doing is helping your conscious mind step to the side so we can communicate with the subconscious directly.

If at any point you want to pause or come back, just raise your right index finger. That is your signal. I will bring you back immediately. Okay? Good. Let us begin.

IF PHYSICAL EP: ARM-RAISING INDUCTION
[See Physical induction sequence below]

IF EMOTIONAL EP: EYE-CLOSURE INDUCTION
[See Emotional induction sequence below]

Physical Induction (if Physical EP)

Raise your arm straight out in front of you. Palm facing your face. Focus on your middle finger.

Take a deep breath in. As you exhale, your fingers begin to separate. Spreading apart. Your hand is moving toward your forehead. With every breath, closer.

The moment your hand touches your forehead, nod your head and close your eyes.

[Wait for hand to touch forehead and nod.]

Each and every time I suggest the words Deep Sleep to you, for the purpose of hypnosis, with your permission, you will enter this state safely and comfortably, quickly, calmly, and deeply, and the physical body will relax.

Deep Sleep.

Hand down to your lap. Five. Going down. Four. Deeper. Three. Past the surface. Two. Comfortable. Safe. One. Zero. Deep Sleep.

Emotional Induction (if Emotional EP)

Find a comfortable position. Take a deep breath. Close your eyes when they feel ready to close. There is no rush.

You may notice your eyelids growing heavier. You may notice a sense of settling. Like the last few minutes before sleep, when the world gets quiet and soft.

Allow yourself to drift. You do not need to try to relax. Just allow it to happen at whatever pace feels right for you.

Each and every time I suggest the words Deep Sleep to you, for the purpose of hypnosis, with your permission, you will allow yourself to enter this state safely and comfortably, at your own pace, and the body and mind will settle.

Deep Sleep.

Drifting deeper now. Five. Letting go a little more. Four. Settling. Three. Safe. Two. Comfortable. One. Zero. Deep Sleep.

Progressive Relaxation (both lanes)

Focus on the top of your head. Allow any tension to soften. Your forehead smooths. Your jaw releases. Your neck and shoulders let go of whatever they have been holding.

Arms. Heavy. Comfortable. Hands open and relaxed.

Chest opens. Breathing slows. Each breath takes you deeper.

Core. Lower back. Hips. Releasing.

Legs. Heavy. Warm. Feet grounded.

Every part of your body is in rest mode now. Safe. Supported. Comfortable.

Safe Space Building (THE ROOM - client designs)

[This safe space becomes THE ROOM for inner child work in Sessions 2-3. The client must design it. NOT a childhood place. A brand new space that belongs to the adult.]

Now I want you to create a space. A room. This room belongs to you and only you. No one can enter without your permission. It is completely safe.

What does this room look like? Take a moment. Notice the colors. The textures. What is on the walls? What kind of light is there? Is there a window? What do you see through it?

[Pause. Let him build it. If he is Physical, prompt with specific sensory questions. If Emotional, let him discover.]

What does it feel like to be in this room? Notice the temperature. Any sounds? Any smells? Is there a chair, a couch, a place to sit that feels exactly right?

Settle into this space. Feel how safe it is. You decide what enters this space and what stays outside. This is your room. Your design. Your space.

This room is now installed. You can return here anytime. In session or on your own. Whenever you need to feel safe, grounded, and in control, close your eyes, take one breath, and you are here.

Subconscious mind, when this room is fully established and you feel completely safe inside it, let a nod happen.

[Wait for nod. This is the first ideomotor check. If no nod comes after 30 seconds, gently prompt: "Take your time. When you feel it, let it happen."]

Good. That nod tells me the subconscious has accepted this space. It is yours now.

Grounding Anchor (for use between sessions)

Before we come back, I am going to give you a tool. A grounding anchor.

Press your thumb and index finger together on your left hand. Feel that pressure. That physical sensation is now linked to this feeling of safety. This room. This calm.

Anytime between now and our next session, if the anxiety starts running the show, or the hypervigilance kicks in, press your thumb and finger together, take one deep breath, and you will feel this calm return. It may not be as deep as right now, but it will be there. A physical anchor to safety.

Subconscious mind, when this anchor is installed, let a nod happen.

[Wait for nod.]

PART 6: COUNT OUT (GENTLE FOR TRAUMA CLIENT)

[Gentle count out. NOT rapid 1-2-3-4-5. Trauma clients need grounding, not abrupt awakening.]

In a moment I am going to bring you back. You will bring with you the safe room and the grounding anchor. Both are installed. Both are permanent.

Zero. Beginning to come back. Slowly.

One. Feeling your body. Your feet on the floor. The chair beneath you.

Two. Taking a deep breath. Energy returning.

Three. More aware now. Feeling refreshed.

Four. Almost back. Eyes ready to open when you are ready.

Five. Eyes open. Fully alert. Grounded. Safe. Welcome back.

Take a moment. How do you feel?

[Let him process. Do NOT rush into clinical debrief. Give him 30-60 seconds of silence if needed. First trance can be emotional, especially for trauma clients.]

PART 7: PHS VERIFY + BASELINES

Let us do a quick test to see how the suggestion took. Raise your arm. Palm facing you. Middle finger. Breathe. Fingers separating. Hand moving toward your face. When it touches, nod. Deep Sleep.

[Snap. Verify PHS is installed. Time the response. Count out 0-1-2-3-4-5.]

SMART Goal Baselines (collect now)

Before we wrap up, I want to get some baseline numbers. These are subjective. There are no wrong answers. Just where you are right now, tonight. On a scale of 1 to 10, where 10 is the best: How comfortable do you feel being yourself right now? Just being in your own skin?

[Record: Self-esteem baseline ____/10]

How easy is it for you to set boundaries or say no when you need to? 1 to 10.

[Record: Assertiveness baseline ____/10]

On a typical day, how much does anxiety run the show? 1 is "barely there," 10 is "completely in control."

[Record: Anxiety baseline ____/10 (NOTE: high number = more anxiety, invert for progress tracking)]

How much do you trust the people closest to you? 1 to 10.

[Record: Trust baseline ____/10]

How often do you feel genuinely at ease, just existing without being on guard? 1 to 10.

[Record: Comfort/hypervigilance baseline ____/10]

And how much does the past still control your present? 1 is "barely at all," 10 is "it runs everything."

[Record: Letting go baseline ____/10 (NOTE: high number = more past control, invert)]

PART 8: HOMEWORK + NEXT STEPS

Here is your homework between now and our next session. Two things.

First: the grounding anchor. Thumb and finger, one deep breath. Use it whenever the anxiety or hypervigilance kicks in. The more you use it, the stronger it gets.

Second: at some point this week, sit quietly for two minutes, close your eyes, and visit the room you built tonight. Walk around it. Notice the details. Strengthen it. This room is going to become an important tool in our work together.

That is it. Two things. No journals, no tracking sheets, no complicated protocols. Just the anchor and the room.

[Keep homework simple for Session 1. This client has therapy fatigue risk. Too much homework = overwhelm = dropout. Build complexity gradually.]

How are you feeling about everything we covered tonight?

[Let him respond. Listen for: relief, surprise, skepticism, emotional processing. All are normal.]

You did really well tonight. The fact that your subconscious accepted the safe space and the grounding anchor on the first session tells me a lot. Your mind is ready for this work.

For our next session, we are going to go deeper. We will use that safe room you built and begin the real therapeutic work. But tonight was the foundation, and it is solid.

Let us schedule session two. I would suggest one to two weeks from now. What works for you?

[Schedule Session 2. Provide Calendly link if needed.]

Last thing. If between now and our next session anything comes up that feels overwhelming, if a memory surfaces or the emotions feel too intense, reach out to me. You have my email. And if it is urgent, call 988, that is the crisis line. You are not alone in this.

Thank you for trusting me with this, T.M. See you at session two.